



Policy

Notifications requirements under the WA Mental Health Act 2014

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Functional subgroup	
Summary	The Mental Health Act 2014 provides that certain events affecting persons under the Act require mental health services to notify the person's personal support person and/or an external person or body such as the Mental Health Advocate or Chief Psychiatrist.
Policy owner	Executive Director Mental Health
Contact	
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NSMHS Standards	2010. 9.3, 9.5
Status	Active

1. Background

1.1. About this document

The Mental Health Act 2014 provides that certain events affecting persons under the Act require mental health services to notify the person's personal support person and/or an external person or body such as the Mental Health Advocate and Chief Psychiatrist

Other events require mental health services, by way of legislation or policy, to notify an external person or body.

Staff within EMHS will ensure they comply with all notifications requirements within the Mental Health Act 2014 and within this policy.

Staff will ensure they have completed the educational requirements re WA Mental Health Act 2014

2. Mandatory Notifications

The parties to be notified are:

- the person's carer, close family member or other personal support person;
- the Mental Health Advocacy Service;
- the Mental Health Tribunal;
- the Mental Impaired Accused Review Board; and
- the Chief Psychiatrist.

Appendix A: Mandatory Notifications explains the relevant parties to be notified following a particular event.

3. Mental Health Tribunal Approvals

Certain treatments require approval by the Mental Health Tribunal before they may lawfully be performed. These treatments are:

- electroconvulsive therapy on a child over the age of 14 who is a voluntary patient;
- electroconvulsive therapy on a child over the age of 14 who is an involuntary patient;
- electroconvulsive therapy on an adult involuntary patient;
- psychosurgery on an adult or on a child over the age of 16.

Refer to [Mental Health Act 2014](#) Sections 195, 196, 198 and 208

4. How External Parties are to be notified of Notifiable Events.

All Mental Health Act forms related to referral, assessment and detainment must be primarily completed in PSOLIS electronically. Should a mental health service receive a paper based form, entry into PSOLIS should occur within 24 hours of receipt into the mental health service.

Where a form is completed electronically, the notification to the relevant external parties (as per Mandatory Notifications in Mental Health Matrix) is automated via PSOLIS.

5. How Carers, Family Member or Personal Support Person is to be Notified of Notifiable Events

All Mental Health Act forms related to referral, assessment and detainment must be primarily completed in PSOLIS electronically. Should a mental health service receive a paper based form, entry into PSOLIS should occur within 24 hours of receipt into the mental health service.

Where a notifiable event occurs, the responsible clinician must ensure that any carer, family member or other personal support person (PSP) hereafter “carer etc.” of a person is notified of the notifiable event as soon as practicable after it occurs.

The requirement to notify is taken to have been complied with if the person responsible makes reasonable efforts to notify the carer etc. until either:

- at least one carer etc. is notified; or
- it is reasonable for the person responsible to conclude that it is not possible for any carer etc. to be notified.

Completion of notification of carer must be indicated in PSOLIS as per local procedure.

Refer to [Mental Health Act 2014](#) Sections 140 and 141

6. Best Interests of a Person

A carer, family member or other personal support person (hereafter “carers etc.”) is not entitled to be notified that any of the following notifiable events has occurred if the medical practitioner or authorised mental health practitioner who makes the order reasonably believes it is not in the best interests of the person for the carer etc. to be notified of the event:

- the making of an order to detain, or to continue to detain, a person to enable them to be taken to an authorised hospital or other place for examination (MHA s. 28); and
- The making of a transport order to take a person to an authorised hospital or other place for examination (MHA s. 29(1)).

A carer etc. is not entitled to be notified that any other notifiable event has occurred if a psychiatrist believes it is not in the best interests of the person for the carer, close family member or other personal support person to be notified of the event.

In deciding what is in a person's best interests, a medical practitioner, authorised mental health practitioner or psychiatrist must have regard to:

- the person's wishes (to the extent that it is practicable to ascertain those wishes);
- the views of the person's ensuring guardian or guardian, parent or guardian, nominated person, carer and close family member, as applicable; and
- any other matter that the person or body considers relevant to making the decision.

Any decision that a carer etc. is not entitled to be notified must be recorded in accordance with the Act.

- If the carer etc. requests to be notified of the event, the decision-maker must:
- advise the carer etc. of the decision to not advise, and the reasons for it; and
- file a record of the advice and give a copy to the patient.

The carer etc. may request a written confirmation of the advice. The decision-maker must comply with the request, file a copy of the written confirmation and give a copy of the written confirmation to the patient.

The decision to not notify a carer etc. may be revoked. If so, the decision-maker must file a record of the revocation and the reasons for it.

If the carer etc. previously requested to be notified of the event, the decision-maker must ensure that as soon as practicable:

- they are notified of the event; and
- a record of when and how they were notified is filed and a copy given to the patient.

Refer to [Mental Health Act 2014](#) Sections 142 (1), 124(2), 7

For further information refer "Recordkeeping".

6.1. Recordkeeping

Staff must ensure they file records relating to notifications in accordance with their obligations under the Mental Health Act 2014.

7. Compliance monitoring and legislative penalties

7.1. Compliance monitoring

Each EMHS Site/Service Executive Director are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

7.2. Legislative obligations

There are legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

A staff member of a mental health service who reasonably suspects that a reportable incident as defined by Section 254 of the Mental Health Act has occurred must report the suspicion to the person in charge of the mental health service or the Chief Psychiatrist.

- Penalty: a fine of \$6000.

A person in charge of a mental health service who becomes aware of the occurrence of a notifiable incident as defined by Section 525 of the Mental Health Act must, as soon as practicable, report the occurrence to the Chief Psychiatrist.

- Penalty: a fine of \$6000.

[Mental Health Act 2014](#) Sections 254(2) and 526 (2)

Reporting Notifiable Incidents - Public Mental Health Services

8. Legislation

- [Mental Health Act 2014](#) Sections 254(2) and 526 (2)
- Mental Health Regulations 2015

9. Additional references

9.1. WA Health policies

9.2. Other

- [Reporting Notifiable Incidents - Public Mental Health Services](#)

9.3. EMHS policies

Nil

10. Appendix A

- Mandatory Notifications in Mental Health with guidelines and Notifiable Incident Report form
- Mandatory Notifications in Mental Health Matrix

Appendix A

1. Notifications in Mental Health Matrix

This document is to be read in conjunction with the Notifications in Mental Health Policy

The notifications matrix on the following pages sets out more than 50 events in respect of a person whereby the Mental Health Service must notify a third person.

The matrix also sets out:

- the person within the mental health service responsible for notifying the third party;
- the relevant section of the Mental Health Act 2014; and
- the approved form to be used in making the notification (if applicable).

Notifications are grouped into sets of events relating to:

- persons referred for examination;
- involuntary inpatients detained at a general hospital;
- involuntary inpatients detained at an authorised hospital or a general hospital;
- patient leave of absence;
- involuntary community patients;
- requests for a further opinion;
- specific treatments and interventions;
- mental health patients who are children;
- interstate transfer of inpatients; and
- other events requiring notification.

1.2 Abbreviations

For the purpose of the notification matrix

- **PSP** means a person's carer, close family member or other personal support person.
- **MHAS** means the Mental Health Advocacy Service established as the Mental Health Advocate under the *Mental Health Act 2014*.
- **MH Tribunal** means the Mental Health Tribunal established under the *Mental Health Act 2014*.
- **MIARB** means the Mentally Impaired Accused Review Board established under the *Criminal Law (Mentally Impaired Accused) Act 1996*.
- **OCP** means the Office of the Chief Psychiatrist

11. Notifications in Mental Health - Appendix A

EMHS Guidelines

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
1. Events relating to persons referred for examination				
1.1 The making of an <u>order to detain</u> , or to continue to detain, a person to enable them to be taken to an authorised hospital or other place for examination	PSP	The practitioner who makes the order	s. 28	3A 3B
1.2 The <u>release</u> of a person who has been detained but has not been taken to an authorised hospital or other place by the end of the period of detention.	PSP	A medical practitioner or authorised mental health practitioner	s. 28(10)	Nil. Document notification in patient's medical record
1.3 The <u>release</u> of a person with respect to whom a referral has expired.	PSP	A medical practitioner or authorised mental health practitioner	s. 28(11)	Nil. Document notification in patient's medical record
1.4 The making of a <u>transport order</u> to take a referred person to an authorised hospital or other place for examination	PSP	The practitioner who makes the order	s. 29	4A
1.5 The <u>release</u> of a referred person where a referral for examination has been revoked.	PSP	The practitioner who revokes the referral	s. 31(6)	Nil. Document notification in patient's medical record

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
1.6 Following psychiatric examination of a person referred to an authorised hospital, the making of an <u>involuntary treatment order</u> (whether inpatient treatment order or community treatment order).	PSP MHAS MH Tribunal MIARB*	The person in charge of the authorised hospital	s. 55(1)(a); s. 55(1)(b)	5A (CTO) 6A (ITO)
1.7 Following psychiatric examination of a person referred to an authorised hospital, the making of an <u>order authorising detention</u> at the authorised hospital for further psychiatric examination.	PSP	The psychiatrist who makes the order	s. 55(1)(c)	3C
1.8 Following psychiatric examination of a person referred to an authorised hospital, the making of an <u>order that the person cannot continue to be detained</u> .	PSP	The psychiatrist who makes the order	s. 55(1)(d)	3E
1.9 Following psychiatric examination of a person referred to a general hospital, the making of an <u>involuntary treatment order</u> (whether inpatient treatment order or community treatment order).	PSP MHAS MH Tribunal MIARB*	The psychiatrist who makes the order	s. 61(1)(a); s. 61(1)(b)	5A (CTO) 6B (ITO)
1.10 Following psychiatric examination of a person referred to a general hospital, the making of an <u>order authorising reception and detention</u> at an authorised hospital for further psychiatric examination.	PSP	The psychiatrist who makes the order	s. 61(1)(c)	3D
1.11 Following psychiatric examination of a person referred to a general hospital, the making of an order that the person cannot continue to be detained.	PSP	The psychiatrist who makes the order	s. 61(1)(d)	3E
2. Events relating to involuntary inpatients detained at a general hospital				

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
2.1 Weekly report on involuntary patient detained at general hospital	OCP	The treating psychiatrist	s. 65(1)	6B
2.2 The making of a transfer order to transfer an involuntary inpatient from a general hospital to an authorised hospital	PSP MH Tribunal (with patient's consent)	The treating psychiatrist	s. 66	4C
2.3 Confirmation of an inpatient treatment order	MH Tribunal (with patient's consent)	The psychiatrist who confirms the order	s. 68(2)	6D
2.4 The release of a person where an inpatient treatment order has not been confirmed by a psychiatrist within 24 hours.	PSP	The person in charge of the authorised hospital	s. 68(6)	Nil. Document notification in patient's
3. Events relating to involuntary inpatients detained at an authorised hospital or a general hospital				
3.1 The making of a continuation order continuing an inpatient treatment order following examination before the end of a detention period.	MH Tribunal (with patient's consent)	The psychiatrist who conducted the examination	s. 89(2)(a)	6C
3.2 The release of an involuntary inpatient where a psychiatrist is satisfied the patient should be treated in the community or released.	PSP	The person in charge of the hospital	s. 89(2)(b); s. 89(2)(c).	Nil. Document notification in patient's
3.3 The making of a community treatment order in respect of an inpatient.	PSP MHAS MH Tribunal MIARB*	The psychiatrist who makes the order	s. 90(1)(a)	5A
3.4 The making of an order revoking an inpatient treatment order.	PSP MHAS MH Tribunal MIARB*	The psychiatrist who makes the order	s. 90(1)(b)	6A

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
3.5 The making of a <u>transfer order</u> transferring an involuntary inpatient between authorised hospitals.	PSP MH Tribunal (with patient's consent)	The psychiatrist who makes the order	s. 91(2)	4C
3.6 The <u>expiry</u> of an inpatient treatment order.	PSP MHAS MH Tribunal MIARB*	The person in charge of the hospital at which the involuntary inpatient was being detained	s. 93(4)	Nil. Document notification in patient's medical notes
4. Events relating to patient leave of absence				
4.1 The <u>absence without leave</u> of a person from a hospital or other place.	PSP	The person in charge of the hospital or other place	s. 97	Nil. Document notification in patient's medical notes
4.2 The making of an order <u>granting</u> an involuntary inpatient leave of absence.	PSP	The psychiatrist who makes the order	s. 105	7A

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
4.3 The making of an order <u>extending or varying</u> an involuntary inpatient's leave of absence.	PSP	The psychiatrist who makes the order	s. 106	7B
4.4 The making of an order <u>cancelling</u> an involuntary inpatient's leave of absence.	PSP	The psychiatrist who makes the order	s. 110	7C
5. Events relating to involuntary community patients				
5.1 The making of an <u>inpatient treatment order</u> in respect of an involuntary community patient following monthly Examination.	PSP MHAS MH Tribunal MIARB*	The supervising psychiatrist	s. 120(2)(a)	6A
5.2 The making of an order revoking a community treatment order in respect of an involuntary community patient following monthly examination.	PSP MHAS MH Tribunal MIARB*	The supervising psychiatrist	s. 120(2)(b)	5A
5.3 The making of a continuation order continuing a community treatment order.	MH Tribunal (with patient's consent)	The supervising psychiatrist	s. 121(1)	5B
5.4 The making of an order varying a community treatment order.	MH Tribunal (with patient's consent)	The supervising psychiatrist	s. 122(1)	5C
5.5 The making of an inpatient treatment order in respect of an involuntary community patient at any time the community treatment order is in force.	PSP MHAS MH Tribunal MIARB*	The supervising psychiatrist	s. 123(1)(a)	6A
5.6 The making of an order revoking a community treatment order in respect of an involuntary community patient at any time the community treatment order is in force.	PSP MHAS MH Tribunal MIARB*	The supervising psychiatrist	s. 123(1)(b)	5A

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
5.7 Confirmation of an inpatient treatment order made in respect of an involuntary community patient	MH Tribunal (with patient's consent)	The psychiatrist who confirms the order	s. 124(2)	6D
5.8 The release of an involuntary community patient where an inpatient treatment order has not been confirmed within 24 hours	PSP	The person in charge of the authorised hospital	s. 124(6)	Nil. Document notification in patient's medical record
5.9 The making of a transport order to transport an involuntary community patient to a specified place.	PSP	The practitioner who makes the order	s. 129(2)	4A
5.10 The release of an involuntary community patient who has not been treated, and in respect of whom an order has not been made, within six hours of being received at a place in compliance with an order to attend.	PSP	The person in charge of the place	s. 130(4)	Nil. Document notification in patient's medical record
5.11 The making of an inpatient treatment order in a general hospital in respect of an involuntary community patient where the patient is in breach of a community treatment order.	PSP MHAS MH Tribunal MIARB*	The supervising psychiatrist	s. 131(2)(a)	6B
5.12 The making of an order revoking a community treatment order in respect of an involuntary community patient where the patient is in breach of a community treatment order.	PSP MHAS MH Tribunal MIARB*	The supervising psychiatrist	s. 131(2)(b)	6B
5.13 The variation of a community treatment order by transferring responsibility as supervising psychiatrist to another psychiatrist.	MH Tribunal (with patient's consent)	The supervising psychiatrist who transfers responsibility or the Chief Psychiatrist or a person authorised by the Chief Psychiatrist under the	s. 135	5C

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
5.14 The variation of a community treatment order by transferring responsibility as treating practitioner to another practitioner.	MH Tribunal (with patient's consent)	The supervising psychiatrist who transfers responsibility	s. 137	5C
6. Events relating to requests for a further opinion				
6.1 The refusal to comply with a request for a further opinion about treatment on the grounds that it is not warranted.	OCP	The patient's psychiatrist	s. 183(3)	Nil. Document notification in patient's medical record
6.2 The reconsideration of a decision to provide treatment upon Chief Psychiatrist request.	OCP	The patient's psychiatrist	s. 184	Nil. Document notification in patient's medical record
7. Events relating to specific treatments and interventions				
7.1 <u>Provision of statistics</u> about electroconvulsive therapy.	OCP	The person in charge of the mental health service	s. 201(3)	13
7.2 The provision of <u>emergency psychiatric treatment</u> .	OCP MIARB*	The medical practitioner who provides the treatment.	s. 204	9A
7.3 The performance of <u>psychosurgery</u> .	OCP MIARB*	The patient's psychiatrist.	s. 209	Nil. Document notification in patient's medical record
7.4 The release of a person from <u>seclusion</u> .	OCP MIARB*	The treating psychiatrist (if applicable) or the person in charge of the authorised hospital where the person was secluded.	s. 224	11B – 11G
7.5 The release of a person from <u>restraint</u> .	OCP MIARB*	The treating psychiatrist (if applicable) or the person in charge of the authorised hospital where the person was restrained.	s. 240	10A – 10I

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
7.6 The provision of <u>urgent non-psychiatric treatment</u> to an involuntary inpatient or a mentally impaired accused detained at an authorised hospital.	PSP OCP MIARB*	The person in charge of the authorised hospital	s. 242	9B
8. Events relating to mental health patients who are children				
8.1 <u>Admission of a child</u> to a mental health service that does not ordinarily provide treatment or care to children.	Chief Psychiatrist	The person in charge of the mental health service	s. 303(3)	Nil. Document notification in patient's medical record
8.2 The provision of <u>off-label treatment</u> to a child who is an involuntary patient.	OCP	The patient's psychiatrist	s. 304	Nil. Document notification in patient's medical record
9. Events relating to interstate transfer of inpatients				
9.1 The making of a <u>transfer order</u> to transfer an inpatient (whether detained at, or absent without leave from, a Western Australian hospital) to an interstate mental health service.	PSP MH Tribunal (with patient's consent)	The person in charge of the hospital who makes the transfer order	s. 555(1)	4D
9.2 The making of a <u>transfer approval order</u> approving the transfer of an interstate inpatient (whether detained at, or absent without leave from, an interstate mental health service) to a Western Australian hospital.	PSP MH Tribunal (with patient's consent)	The person in charge of the hospital who makes the transfer approval order	s. 557(1)	4E
10. Other events requiring notification				
10.1 The making of a decision that a personal support person is not entitled to be notified of a notifiable event.	MHAS	The practitioner or psychiatrist who makes the decision.	s. 142(3)	Nil. Document notification in patient's medical record

Event	Party to be notified	Person responsible	Section of MHA 2014	Approved form
10.2 The making of an order prohibiting a patient from exercising, or limiting the extent to which a patient can exercise, a right of freedom of lawful communication.	PSP MHAS	The psychiatrist who makes the order.	s. 262(6)(b); s. 262(10)	12C
10.3 The occurrence of a “notifiable incident” as defined by s. 525 of the MHA2014.	OCP	The person in charge of the mental health service	s. 526(1)	Notifiable Incident Report forms

The Mentally Impaired Accused Review Board is only required to be notified where an involuntary inpatient is a mentally impaired accused as defined by s. 23 of the *Criminal Law (Mentally Impaired Accused) Act 1996*.